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Correct Answer

Next C

Practice Exam - 1

Question 28 of 117



A 76-year-old man with coronary heart disease, prior inferior wall myocardial infarction 2 years ago and right coronary artery stenting has worsening exertional dyspnea. Current medications: sacubitril valsartan 49/51 mg bid, carvedilol 25 mg bid, empagliflozin 10 mg, eplerenone 25 mg, furosemide 20 mg, clopidogrel 75 mg, rosuvastatin 20 mg. Exam: BP 118/72, pulse 68 bpm, no JVD, clear lungs, regular heart rhythm with II/VI holosystolic murmur, no edema. Labs: creatinine 2.0 mg/dL, potassium 5.0 meq/L, LDL 68, AST 28, T. Bili 1.0. Echocardiogram shows LVEDD 6.8 cm, LVEF 38%, severe mitral regurgitation with EROA 0.44 cm². He returns to the office one month after increasing sacubitril/valsartan to 97/103 mg bid. He is able to walk only one block before become dyspneic. Repeat echocardiogram shows an EROA 0.4 cm².

Which of the following is the best next step in his management?

- A. Increase eplerenone 50 mg daily
-  B. Increase carvedilol 50 mg twice daily
- C. Refer for durable left ventricular assist device
-  D. Refer for transcatheter edge-to-edge mitral valve repair
- E. Start vericiguat 2.5 mg daily

Commentary

Testing Point: The Board-certified AHFTC specialist should recognize that persistent severe mitral regurgitation following optimization of GDMT is an indication for transcatheter intervention.

Correct Answer: D. Refer for transcatheter edge-to-edge mitral valve repair

